

An Integrative Behavioral Approach to Therapy

PSY219: Clinical Psychology

Jennèa Jeter

Final Essay

1. Theoretical Approach

As I reflect on my development as a clinician, I identify primarily as an Integrative Behavioral Therapist. My approach focuses on the “Third Wave” of behavioral psychology, utilizing Acceptance and Commitment Therapy (ACT) as my primary framework and supported by the foundational principles of Behavior Therapy (BT). I view human suffering not merely as the presence of pain, but as the result of "cognitive fusion" and the struggle to control or avoid inevitable internal experiences. I believe that distress arises when we treat our internal narratives as literal truths rather than as aspects of ongoing experience.

I bring these models together by using Behavior Therapy to look at learned behaviors shaped by a client’s environment, and ACT to help clients relate to their thoughts in new ways. Instead of trying to get rid of painful thoughts, I help clients notice when they get stuck in them and start treating them as facts (PTT, p. 82). For example, if someone thinks “I am a failure,” they might stop trying, withdraw from things they care about, and feel more hopeless. Exercises to help clients notice these thoughts, recognize them as just thoughts, and choose actions that align with their values, such as applying for a job or reaching out for support, even when the thought is still present. This way, clients can change their actions without having to debate their thoughts.

I also draw on concepts from other models, such as the Internal Working Models (IWMs) from Brief Dynamic Therapy, which help explain how early relationships create a blueprint for the future (PTT, p. 32). However, I use ACT rather than psychoanalysis to help clients change their relationship to these internal blueprints. I recognize that I cannot simply remove the client’s history or “fix” their thoughts,

because “there is no healthy eraser” for what we may have learned (PTT, p. 86). Instead, my focus is to help clients develop psychological flexibility (PTT, p. 81) and flexible behavioral repertoires (PTT, p. 15), allowing them to persist in behaviors that serve their values even when things are difficult.

2. Evaluation of the Approach

I know that a clinician can't erase a client's painful past or the bigger problems they face, but ACT and BT are helpful because they focus on what works, not on chasing constant happiness. These methods don't eliminate tough thoughts, but they change how those thoughts affect clients (PTT, p. 82). For example, if someone is afraid of social rejection and avoids social situations, I might use exposure techniques to help them slowly face these situations, feel the anxiety, and stay present. Over time, this helps clients see that their difficult thoughts are manageable and don't have to control what they do. Exposure techniques that help clients face their fears or uncomfortable feelings (PTT, p. 59) can show them they can handle discomfort without being stuck by it.

Another major benefit of this approach is its focus on empowerment. Behavior Therapy sees all behavior as learned and serving a purpose (PTT, p. 19). If someone learned an unhelpful behavior that helped them survive before, they can also learn better ways to cope. This helps take away shame and puts the focus on a person's history and situation, not on their character.

Every approach has its limits. Sometimes therapy can become too technical or feel more like an intellectual exercise, making it less effective. If a therapist uses too many behavioral or mindfulness techniques before building a strong relationship, clients may feel misunderstood or disconnected. Empathy and support are essential for helping clients engage in therapy and benefit from it.

Additionally, as an individual, there is a distinct limit to what I can “fix” as systemic dysfunction provides the boundary of what is possible. While ACT and BT are excellent for building personal resilience, they cannot directly dismantle the upstream forces. Poverty, discrimination, or lack of access to social services

often cause distress (Wilcox, 2022, p. 4), and focusing solely on the individual's reaction to their environment risks failing to address the toxicity of the environment itself.

3. Multicultural Competence

My integrative behavioral approach facilitates multicultural competence by shifting the focus from “truth” to “function.” In traditional Cognitive Therapy, a therapist might challenge the validity of a client's thoughts. However, Multicultural CBT (MCBT) teaches us that when a client's negative belief is based on valid upstream forces, challenging the validity of that thought can be inappropriate and potentially harmful (Graham et al., 2013, p. 106).

The client's suffering may not just be an individual issue but a reaction to structural forces (Wilcox, 2022, p. 4). Within the ACT framework, I can recognize a client's reality while also asking if the way they handle a thought is helpful. For example, if a client believes “the system is rigged against me,” that might be true based on history and facts, so it is not a cognitive distortion. As a therapist, I can ask if holding onto that thought right now helps them or keeps them stuck. This way, I can respect their experience with oppression and help them find agency within those constraints.

4. Personal Qualities

I believe a therapist's orientation should align with who they are as a person. Personally, I believe deeply in determinism. If I were you, looked like you, and was raised like you, I'd be you. That mindset aligns with Behavior Therapy, which views our actions as the product of our learning history and environment rather than as random choices for which we are to blame (PTT, p. 19).

Determinism allows for radical empathy, as I view people not as “flawed” characters, but as individuals doing the best they can given their specific context and history. I don't judge people for where they ended up; I investigate the road they traveled. This quality makes me well-suited for a model that seeks to understand the function of behavior rather than judging it.

Furthermore, I deeply value the concept of Collaborative Empiricism (PTT, p. 66), where therapist and client work together to investigate the client's experience. I possess a strong "desire to listen and learn," and believe therapy should be a reciprocal process where I learn about humanity through the client. I am not an expert on a client's life, but instead a partner in figuring out how they can navigate their specific constraints.

My preference for routine and structure also aligns with the systematic nature of behavioral interventions, allowing me to provide a stable "secure base" (PTT, p. 29), from which clients can explore change. I genuinely want to know how humans work, and I believe that by understanding the mechanics of our conditioning, we can find more autonomy.

5. Personal Challenges

Like any field, I know I will face personal challenges as I grow as a therapist. The biggest challenge is managing allostatic load, which is the stress and emotional strain that can build up from hearing about trauma and injustice (MT, p. 138). Both ACT and BT stress the importance of psychological flexibility and self-care routines to maintain well-being during ongoing stress. If I neglect these practices, I could risk burnout, so keeping up with my routines and making self-care a priority are not just personal goals but also ways to model the flexibility and resilience I want to encourage in my clients.

I must also remain vigilant about my own implicit bias and the risk of committing microaggressions. I believe anyone can be racist, sexist, or classist, regardless of their own background. The challenge will be maintaining cultural humility, an ongoing process of self-reflection and self-critique (MT, p. 66). To address microaggressions when they occur, I plan to actively seek feedback from clients and colleagues, openly acknowledge any harm caused, and initiate conversations when appropriate. Making mistakes is human, but failing to analyze and apologize for them is unprofessional and invalidating.

Finally, I struggle with being too much of a therapist to people around me when they don't need it. My challenge will be practicing therapeutic humility, trusting that people are the experts in their own lives and

not needing to fix every person I meet. I need to know when to be a therapist versus when to just be a friend, partner, or stranger.

Conclusion

My integrative behavioral approach aims to balance the impact of larger systemic forces with the real possibility for individual change. By combining the nurturing principles of Behavior Therapy with the flexibility of ACT, I can help clients build practical skills to handle challenges with more agency and resilience. Rather than attempting to eliminate adversity, my role is to guide clients in accepting their circumstances, cultivating insight, and implementing functional behavior changes that move them toward personally meaningful lives. Overall, my goal is to facilitate clients' capacity to withstand and manage pain for long-term growth and well-being.